Dillingham Census Area, Alaska

Review level unavailable

Gap review score **24.3** / 100

Why it surfaced

Dillingham Census Area has a published gap review score of 24.3. This county is outside the contiguous tick scope of the current release, so a review level is not assigned. No county-level published Lyme disease case or pathogen records are in the current datasets. Local information may help interpret these gaps. This score is not a measure of individual risk.

County data: Six review factors

These factors summarize the published data used in the Atlas. They do not show how much each factor contributed to the county's overall score.

1	**Published Lyme case data**	No county-level 2023 Lyme disease case count was published. A missing record is not treated as zero cases.
2	**Tick presence**	The published tick status is Unknown. This is not a finding of tick absence.
3	***Borrelia burgdorferi* detection**	No county-level *Borrelia burgdorferi* detection record is available; this does not establish pathogen absence.
4	**Social vulnerability**	The 2022 social vulnerability percentile is 89.3. Higher values indicate greater vulnerability on the source index. This is context for access and outreach, not proof of underreporting.
5	**Health insurance coverage**	The published percentage uninsured is 18.1%. This is context for healthcare access, not proof that cases are being missed.
6	**Rurality**	USDA Rural-Urban Continuum Code 9 (2023): completely rural or less than 2,500 urban population, not adjacent to a metro area. Code 1 is the most metropolitan and 9 is the most rural. This is context for follow-up planning, not a tick-habitat or underreporting measure.

Interpreting the data

Lyme disease data are grouped by county of residence, not necessarily the location of exposure. These values describe published surveillance, not individual risk. A missing published case count is not an observed zero. Missing pathogen data is not a documented negative finding. The percentage uninsured is context; the score uses the uninsured percentile.

Observation periods: Lyme case records 2023; social vulnerability 2022; rurality 2023.

Suggested follow-up

This county is outside the contiguous tick scope of the current release, so a review level is not assigned. These topics are for checking published records. They are not a scored recommendation or an instruction to escalate surveillance.

1	**Human disease surveillance**	Review the published human surveillance information that is available. Check whether additional county case information is available through local records, including suppressed or unallocated reports. A missing published count is not a finding of zero cases.
2	**Vector and pathogen surveillance**	Review the published tick and pathogen information that is available. The published tick status is Unknown. This is not a finding of tick absence. Check whether newer local records change how that status should be read.
3	**Clinical and laboratory reporting**	Maintain awareness of existing reporting and laboratory surveillance procedures. This is not a finding that laboratories failed to report.
4	**Community context and outreach**	Use available community-context measures only as context for access and outreach. Use social vulnerability, health insurance coverage, and rural-urban classification as context for access and outreach. These measures are not proof of transmission or underreporting.
5	**Coordination and documentation**	Document which data are missing and identify possible sources for clarification. These notes support local verification. Atlas does not assign an owner, a deadline, or an escalation.

Questions before acting

1. Why is a follow-up category not assigned for this county?
2. Which published evidence is missing, limited, or outside the supported scope?
3. Are relevant state or local records available to clarify the evidence without assuming a particular response level?
4. Should these limitations be documented for routine review rather than treated as an assigned priority?

Data sources and more information

Definitions, data sources, and methods are available on the Atlas website.

Lyme surveillance data | Tick and pathogen data | Clinical testing guidance | Social vulnerability index | Health insurance data | Rural-urban classification | County review methodology | MRLC Annual NLCD 2025 reviewed demo cohort (C1V2-2025) | USDA ERS Rural-Urban Continuum Codes (2023) | CDC/ATSDR Social Vulnerability Index (2022 (2018-2022 ACS)) | CDC Lyme surveillance (2023)

Document details

County: Dillingham Census Area, Alaska
County identifier (FIPS): 02070
Published gap review score: 24.3
Review category: Review level unavailable

Data reference years

Lyme case records: 2023
Social vulnerability: 2022
Rurality: 2023

Document information

Report date: 2026-10-21
Template version: county data brief v3.0
Website: onehealthatlas.org
Data release: governed-2026-10-15-january-climate-prod

Data methodology: semantic-2.0.0
Score methodology: surveillance-gap-score-v1.1; smooth k=50
Generated: 2026-10-21 04:08 UTC
Source vintages: MRLC Annual NLCD 2025 reviewed demo cohort (C1V2-2025); USDA ERS Rural-Urban Continuum Codes (2023); CDC/ATSDR Social Vulnerability Index (2022 (2018-2022 ACS)); CDC Lyme surveillance (2023); CDC ArboNET tickborne pathogen status (through 2025-12-31); CDC ArboNET county tick status (through 2025-12-31)
Published release only. What-if score settings are not applied.
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